Referral packet F17: readable copy

Source: F17.pdf (fictional), 12 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

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10	ECG report	ecg	good	
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Fax cover sheet

Lakeside Valley Hospital (fictional)
Case Management Department
FAX TRANSMITTAL
To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Gregory Ambrose, MSW
Phone: 555-0171
Date: 09/30/2026 10:05
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]
Message
63F CHF exacerbation, diuresed, now on PO diuretic. Independent.
Anticipated d/c 10/01. Please review.
CONFIDENTIALITY NOTICE
This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Lakeside Valley Hospital (fictional)

Case Management Department

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Urgent **[X]** For Review **[X]** Please Reply **[X]**

Message

63F CHF exacerbation, diuresed, now on PO diuretic. Independent.
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-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME
Recuperative Care Referral Form (RC0725-SH)
Referring Party
Hospital: Lakeside Valley Hospital (fictional)
Department/Floor: 5 Telemetry
Social Worker/Case Manager/Nurse: Gregory Ambrose, MSW
Phone: 555-0171
Email: gambrose@lakesidevalley.example
Nursing Station: 555-0172
Authorized By: Dr. N. Ferreira
Patient Information
Name: Rosa Doe
MRN: LV-2287415
Birth Date: 06/03/1963
Gender: F
Ethnicity: Hispanic/Latino
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: Spanish/English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]
Hospital Admission Details
Chief Complaint/Admitting Diagnosis: Acute on chronic heart failure exacerbation
Admit Date: 09/26/2026
Any Wounds: Yes [ ] No [X]
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No [X]
PPD/TB Test or Chest X-Ray: Date: 09/26/2026 Result: CXR - pulm edema, no TB findings
History of Health & Living Situation
O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [ ] No [X]
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]
Behavioral and Mental Health Status
Any limitations or challenges: No [X] Yes [ ]

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

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Department/Floor: 5 Telemetry
Social Worker/Case Manager/Nurse: Gregory Ambrose, MSW
Phone: 555-0171
Email: gambrose@lakesidevalley.example
Nursing Station: 555-0172
Authorized By: Dr. N. Ferreira

Patient Information

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Gender: F
Ethnicity: Hispanic/Latino
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: Spanish/English
Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Acute on chronic heart failure exacerbation
Admit Date: 09/26/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No **[X]**
PPD/TB Test or Chest X-Ray: Date: 09/26/2026 Result: CXR - pulm edema, no TB findings

History of Health & Living Situation

O2: Yes [ ] No **[X]**
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes [ ] No **[X]**
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes [ ] No **[X]**
Active Infection: Yes [ ] No **[X]**
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes [ ] No **[X]**
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other:

Length of Stay Authorized

Days: 14

Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

Referring Party Comments

Cardiac: EF normal. Troponin negative x3.
Diuresed 6 L, on PO furosemide. Needs low-sodium diet and daily weights.

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CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal
Member
Member Name: Rosa Doe
DOB: 06/03/1963
Member ID: OCH-40877152
Referring Provider
Facility: Lakeside Valley Hospital (fictional)
Clinical
Primary Diagnosis: Acute on chronic heart failure, unspecified
ICD-10: I50.9
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 10/01/2026
Requested Days: 14
Admit Date: 09/26/2026
Plan Use Only
Status: Pending review

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Rosa Doe
DOB: 06/03/1963
Member ID: OCH-40877152

Referring Provider

Facility: Lakeside Valley Hospital (fictional)

Clinical

Primary Diagnosis: Acute on chronic heart failure, unspecified
ICD-10: I50.9
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 10/01/2026
Requested Days: 14
Admit Date: 09/26/2026

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Lakeside Valley Hospital (fictional) - PATIENT FACE SHEET
Patient: Rosa Doe
MRN: LV-2287415
Account #: 99287415
DOB: 06/03/1963
Sex: Female
Preferred Language: Spanish (fluent English)
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)
Encounter
Admit Date/Time: 09/26/2026 16:30 (ED arrival 09/26/2026 11:20)
Admit Type: Inpatient
Unit/Bed: 5T-522
Attending: Nuno Ferreira, MD
Insurance
Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self
Emergency Contact
None listed

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Lakeside Valley Hospital (fictional) - PATIENT FACE SHEET

Patient: Rosa Doe
MRN: LV-2287415
Account #: 99287415
DOB: 06/03/1963
Sex: Female
Preferred Language: Spanish (fluent English)
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 09/26/2026 16:30 (ED arrival 09/26/2026 11:20)
Admit Type: Inpatient
Unit/Bed: 5T-522
Attending: Nuno Ferreira, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

Emergency department note

EMERGENCY DEPARTMENT PROVIDER NOTE

Arrival: 09/26/2026 11:20, walk-in

HPI

63 y/o F, unhoused (staying in car of friend, now outside), with "heart failure" diagnosed 2024 at outside hospital (records not available). Ran out of furosemide ~3 wks ago. Leg swelling, orthopnea, DOE x 1 wk.

Exam

T 98.1F HR 98 BP 162/94 RR 22 SpO2 91% RA -> 95% 2L NC
JVD to angle of jaw, bibasilar crackles, 3+ pitting edema bilat LE to knees

Data

BNP 1,640 pg/mL H
Troponin I (hs) 11:45: 14 ng/L (ref <17)
Na 134, K 3.9, Cr 1.1
ECG: sinus rhythm 96, LVH, no acute ST changes
CXR: cardiomegaly, interstitial edema, small bilat effusions

ED Course

Furosemide 40 mg IV x1. Admit to telemetry.
Signed: L. Brandt, MD 09/26/2026 15:40

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 5: Emergency department note

other | orientation normal | legibility good

EMERGENCY DEPARTMENT PROVIDER NOTE

Arrival: 09/26/2026 11:20, walk-in

HPI

63 y/o F, unhoused (staying in car of friend, now outside), with "heart failure" diagnosed 2024 at outside hospital (records not available). Ran out of furosemide ~3 wks ago. Leg swelling, orthopnea, DOE x 1 wk.

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-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL
Date: 09/26/2026
CC
Leg swelling and shortness of breath
HPI
63 y/o F with heart failure (type and EF unknown - outside records requested, not yet received), HTN, presents with volume overload after running out of furosemide. No chest pain. Single hs-troponin in ED within normal limits.
PMH
- Heart failure (dx 2024, outside hospital)
- Hypertension
- Hyperlipidemia
Meds PTA
- Furosemide 40 mg daily (not taking x 3 wks)
- Lisinopril 10 mg daily (not taking x 3 wks)
Allergies
Sulfa (rash)
Social
Never smoker. No EtOH or drug use.

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 6: History and Physical - page 1

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date: 09/26/2026

CC

Leg swelling and shortness of breath

HPI

63 y/o F with heart failure (type and EF unknown - outside records requested, not yet received), HTN, presents with volume overload after running out of furosemide. No chest pain. Single hs-troponin in ED within normal limits.

PMH

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Meds PTA

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- Lisinopril 10 mg daily (not taking x 3 wks)

Allergies

Sulfa (rash)

Social

Never smoker. No EtOH or drug use.

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Exam
T 98.4F HR 94 BP 158/90 RR 20 SpO2 95% 2L NC
JVD +, crackles bilat bases, 3+ edema bilat LE
AOx4
Assessment/Plan
1. Acute on chronic HF exacerbation, EF unknown - furosemide 40 mg IV BID, strict I/O, daily weights, fluid restriction 1.5 L, 2 g Na diet
2. TTE ordered to evaluate EF
3. HTN - resume lisinopril when Cr stable
4. Outside records request (prior echo)
Signed: Nuno Ferreira, MD 09/26/2026 18:00

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Page 7: History and Physical - page 2

h_and_p | orientation normal | legibility good

Exam

T 98.4F HR 94 BP 158/90 RR 20 SpO2 95% 2L NC
JVD +, crackles bilat bases, 3+ edema bilat LE
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1. Acute on chronic HF exacerbation, EF unknown - furosemide 40 mg IV BID, strict I/O, daily weights, fluid restriction 1.5 L, 2 g Na diet
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Signed: Nuno Ferreira, MD 09/26/2026 18:00

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5
Date: 09/30/2026 08:15
S: Breathing at baseline, sleeping flat. Edema much better.
O: T 98.2F HR 76 BP 128/78 RR 16 SpO2 96% RA. Wt 71.4 kg (admit 77.6 kg). Trace LE edema. AOx4.
A/P:
1. Acute on chronic HF exacerbation - euvolemic. Last IV furosemide 09/28 18:00. Transitioned to furosemide 40 mg PO daily 09/29 08:00; weight and BP stable on PO regimen.
- TTE NOT DONE this admission (echo lab backlog; pt not prioritized as stable). EF remains unknown. Outside records request sent 09/27 - no response yet. Recommend outpatient TTE.
- Troponin: single ED value only, not repeated (no chest pain).
2. HTN - lisinopril 10 mg PO daily resumed 09/28, BP controlled.
3. Off O2 since 09/28.
4. Dispo - medically stable for d/c 10/01 pending placement.
Signed: Nuno Ferreira, MD

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Page 8: Hospitalist progress note

progress_note | orientation normal | legibility good

HOSPITALIST PROGRESS NOTE - HD#5

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2. HTN - lisinopril 10 mg PO daily resumed 09/28, BP controlled.
3. Off O2 since 09/28.
4. Dispo - medically stable for d/c 10/01 pending placement.
Signed: Nuno Ferreira, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing shift note

NURSING SHIFT NOTE
Date: 09/30/2026 06:00
Pt AOx4. Independent with grooming, dressing and toileting at bedside. Continent. Up to bathroom independently, no device. Shower not attempted this admission.
Self-administers PO meds; teach-back done on furosemide and daily weights.
Room air. Telemetry: sinus rhythm 70s, no events.
Weight 71.4 kg standing scale.
Signed: M. Sato, RN

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 9: Nursing shift note

nursing_note | orientation normal | legibility good

NURSING SHIFT NOTE

Date: 09/30/2026 06:00
Pt AOx4. Independent with grooming, dressing and toileting at bedside. Continent. Up to bathroom independently, no device. Shower not attempted this admission.
Self-administers PO meds; teach-back done on furosemide and daily weights.
Room air. Telemetry: sinus rhythm 70s, no events.
Weight 71.4 kg standing scale.
Signed: M. Sato, RN

-- FICTIONAL DEMO RECORD - not a real patient --

ECG report

12-LEAD ECG REPORT
Date/Time: 09/26/2026 11:30
Rate 96 bpm. Rhythm: sinus rhythm
PR 168 ms, QRS 98 ms, QTc 452 ms
Left ventricular hypertrophy by voltage criteria
No acute ST-T changes. No prior ECG for comparison.
Read by: R. Castellanos, MD (Cardiology)

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Page 10: ECG report

ecg | orientation normal | legibility good

12-LEAD ECG REPORT

Date/Time: 09/26/2026 11:30
Rate 96 bpm. Rhythm: sinus rhythm
PR 168 ms, QRS 98 ms, QTc 452 ms
Left ventricular hypertrophy by voltage criteria
No acute ST-T changes. No prior ECG for comparison.
Read by: R. Castellanos, MD (Cardiology)

-- FICTIONAL DEMO RECORD - not a real patient --

Labs and vitals

LABS

09/26 | BNP 1,640 H | Troponin I (hs) 14 ng/L (ref <17) | Na 134 | K 3.9 | Cr 1.1 | Hgb 12.1
09/28 | Na 137 | K 3.6 | Cr 1.2 | Mg 2.0
09/30 | Na 138 | K 4.1 | Cr 1.1 | BNP 610 H
Troponin repeat: not ordered
Echocardiogram (TTE): not performed this admission
09/26 MRSA nares PCR: Negative
09/30 07:00 SARS-CoV-2 rapid antigen: Negative
09/26 CXR: cardiomegaly, interstitial edema, small bilat effusions

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
09/29 08:00	98.0	78	130/80	16	96%	RA
09/29 14:00	98.4	80	126/78	16	97%	RA
09/29 20:00	98.2	74	124/76	16	96%	RA
09/30 02:00	97.9	70	122/74	14	96%	RA
09/30 08:15	98.2	76	128/78	16	96%	RA

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Page 11: Labs and vitals

labs | orientation normal | legibility good

LABS

09/26 | BNP 1,640 H | Troponin I (hs) 14 ng/L (ref <17) | Na 134 | K 3.9 | Cr 1.1 | Hgb 12.1
09/28 | Na 137 | K 3.6 | Cr 1.2 | Mg 2.0
09/30 | Na 138 | K 4.1 | Cr 1.1 | BNP 610 H
Troponin repeat: not ordered
Echocardiogram (TTE): not performed this admission
09/26 MRSA nares PCR: Negative
09/30 07:00 SARS-CoV-2 rapid antigen: Negative
09/26 CXR: cardiomegaly, interstitial edema, small bilat effusions

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
09/29 08:00 | 98.0 | 78 | 130/80 | 16 | 96% | RA
09/29 14:00 | 98.4 | 80 | 126/78 | 16 | 97% | RA
09/29 20:00 | 98.2 | 74 | 124/76 | 16 | 96% | RA
09/30 02:00 | 97.9 | 70 | 122/74 | 14 | 96% | RA
09/30 08:15 | 98.2 | 76 | 128/78 | 16 | 96% | RA

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list

DISCHARGE MEDICATION LIST (DRAFT)
Patient: Rosa Doe MRN: LV-2287415
- Furosemide 40 mg PO daily
- Lisinopril 10 mg PO daily
- Potassium chloride 10 mEq PO daily
- Atorvastatin 40 mg PO nightly
Diet: 2 g sodium, 1.5 L fluid restriction. Daily weights; call PCP for gain > 2 lb/day or 5 lb/week.
Follow-up: PCP within 7 days; outpatient TTE to be scheduled

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 12: Discharge medication list

med_list | orientation normal | legibility good

DISCHARGE MEDICATION LIST (DRAFT)

Patient: Rosa Doe MRN: LV-2287415

- Furosemide 40 mg PO daily
- Lisinopril 10 mg PO daily
- Potassium chloride 10 mEq PO daily
- Atorvastatin 40 mg PO nightly

Diet: 2 g sodium, 1.5 L fluid restriction. Daily weights; call PCP for gain > 2 lb/day or 5 lb/week.

Follow-up: PCP within 7 days; outpatient TTE to be scheduled

-- FICTIONAL DEMO RECORD - not a real patient --